

Michigan
SYSTEM PROTOCOL
QUALITY IMPROVEMENT PROGRAM

Initial Date: September 2004

Revised Date: 6/8/2017

Section: 8-27

Quality Improvement Policy

Purpose: The purpose of this policy is to establish the requirement for a defined Quality Improvement process within the Medical Control Authority (MCA) and with agencies holding medical control privileges. This policy provides a means for evaluation and improvement of protocol and EMS system components and design.

I. Confidentiality Assurance

Information obtained for the purpose of Quality Review will be used to determine if the current protocols in the MCA are being appropriately followed and to improve the protocols and the EMS system. Data is protected under P.A. 270 of 1967, MCL 331.531 to 331.533.

In specific cases where EMS providers may require corrective actions, the emergency medical services personnel names may be given to the agency to address at the agency level.

II. Professional Standards Review Organization

- A. The Professional Standards Review Organization (PSRO) of the MCA is a review entity that is provided information or data regarding the physical or psychological condition of a person, the necessity, appropriateness, or quality of health care rendered to a person, or the qualifications, competence, or performance of a health care provider. The PSRO is a committee established by the MCA for the purpose of improving the quality of medical care and oversight of appropriate protocol compliance within the EMS system.
- B. Agencies shall develop institutional PSROs for the purpose of internal review and improvement. For the purpose of this protocol, PSRO is meant to refer to the PSRO of the MCA.
- C. The MCA's designated PSRO shall perform the duties and functions related to complaints, investigations or quality improvement activities, both prospective and retrospective.
- D. The PSRO may be comprised of members of the board(s), MCA employees and contract staff, EMS agency staff, hospital staff, committee members, and other designated individuals when acting on behalf of, or at the direction of the MCA when performing PSRO tasks.
- E. All Quality Improvement activities shall be performed by the PSRO, and all documents collected for Quality Improvement activities shall be held by the PSRO subject to Michigan's peer review privilege.¹

¹ MCL 331.531 *et seq.*

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III. Data Collection

- A. Electronic Patient Care Reports (EPCR)
The MCA is authorized to obtain access to EPCR originating within their service area; this includes all scene responses, interfacility transfers and critical care transfers. The Medical Control may elect to receive reports on request.
- B. MI-EMSIS Data Collection
 1. Providers and agencies are required to report per **Electronic Records & EMS Information System Protocol and Documentation and Patient Care Records-Procedure Protocol**.
 2. Agencies shall work in cooperation with the MCA, under PSRO, to ensure the quality, consistency and accuracy of data submitted through MI-EMSIS.
 3. The MCA shall maintain access to the MI-EMSIS data and ensure that agencies are accountable for the submission of data.
 4. MI-EMSIS data should be utilized as a tool for the evaluation of performance and function as a driving mechanism for quality improvement.
- C. Other Electronic Data Collection
The MCA is authorized to obtain electronic data and voice recordings from any and all EMS agencies and/or departments, and dispatch agencies with interaction with callers requesting a medical response within the MCA service area. This includes mutual aid responses into the MCA service area. Data will be provided to the MCA's PSRO on a monthly basis or when individual records, recordings and reports are requested. The Medical Control may elect to receive electronic reports on a more frequent schedule.
- D. Ownership of Records
Any documents or data relating to requests for service, records of provided services, records of refused services, dispatch reports and incident reports including all aggregated reports for benchmarking and analysis which are submitted to the PSRO of the MCA, or generated by the PSRO, are privileged. The MCA's PSRO holds ownership of only protected Quality Improvement documents. The submitting agency maintains ownership of any and all original records generated by their agency and personnel.
- E. Incident Report Collection
 1. Incident reports and requests for additional information directed to an individual provider or to an EMS agency/department requested by the MCA/PSRO must be submitted to the MCA/PSRO within 96 hours.
 2. The MCA may establish an online reporting system.

IV. Data Review

- A. Agency PSRO Responsibilities

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- Each agency, or department licensed to provide prehospital care, within the MCA area must develop and maintain a PSRO subgroup that reviews, either through a peer evaluation group or individuals tasked with peer review functions, and conducts audits requested by Medical Control.
- B. Special Studies
All EPCR that include the use of equipment, skills, techniques or procedures that are currently under special study will be reviewed.
 - C. Unusual Occurrences
Any EPCR that are unusual and possibly one-time situations that may serve as a learning tool for other services in the future may be reviewed.
 - D. Problem Identification
 - 1. Potential concerns in patient care may be brought to the attention of the PSRO of the MCA.
 - 2. Topic quality improvement reviews will be performed with results reported to the Medical Control Authority.
 - E. Sentinel Event Reporting
 - 1. The Medical Control Authority may designate specific items that must be reported.
 - 2. Any intervention where it is reasonable to believe that harm to the patient may have occurred must be reported.

VI. Quality Review Criteria

- A. Medical Control Authority Protocols
 - 1. The current protocols in place at the time of the event will be used to review the EPCR selected.
 - 2. Any changes in protocols will not be used for evaluation until the changes are approved and distributed.
- B. Dispatch Policies
The review of the EPCR may address dispatch, location, response time, or mutual aid/multi-agency problems.

VII. Quality Improvement Actions

The PSRO, the Medical Director or his/her designee will determine the severity of the incident and develop an action plan to address the matter. The action plan may include:

- A. Revision of policies/procedures
- B. Remediation of individuals involved
- C. Education recommendations for the system
- D. Referral to Due Process and Disciplinary Procedures Protocol
- E. Modification of clinical privileges
- F. Continued monitoring